Second. If deep involving the fasciæ, use B–C, B–D, and A–D currents; the *positive* upon the affected part, and the negative below to draw the inflammation downward.

Third. If phlegmonous, guard against suppuration, arrest it, and cause re-absorption, if requisite and feasible, by localized use of G–H alternately with A–D, frequently repeated, covering the diseased tract fully with tepid wet cloth, to which apply the negative by a large electrode, tied or held in place; positive on outer edge of the affected space, and a little above.

Fourth. If pus has already formed in sufficient amount to demand discharge, a suitable spot is chosen, and the negative focalized there by means of the wire brush, with its ends gathered into a compact bunch, thus acting as a gradual cautery, which is readily regulated in the extent and depth to which it shall act; the positive above, and at a short distance only from the other pole, using B–D, B–C, and A–D currents. If sloughs form internally, space must be made for their escape. If suppuration be widespread, several openings may be required.

Fifth. If sloughing occur extensively, a fatal issue is inevitable, unless checked immediately. Hence it demands early preventive attention, wherever phlegmonous erysipelas attacks a dense structure, such as the dorsal fascia, either of the thigh or trunk; the course recommended for this form in general being the prophylactic in such case.

The general system, and special organs and functions, must receive careful attention.